

COLUMBIA-SUICIDE SEVERITY RATING SCALE (C-SSRS) — COMPLETE GUIDE

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1. Introduction & Historical Background

1.1 Developers & Origin

The **Columbia-Suicide Severity Rating Scale (C-SSRS)**, also known as the **Columbia Protocol**, was developed by:

Kelly Posner, Ph.D., David Brent, M.D., Christopher Lucas, Ph.D., Madelyn Gould, Ph.D., Barbara Stanley, Ph.D., Gregory Brown, Ph.D., Prudence Fisher, Ph.D., Jill Zelazny, Ainsley Burke, Maria Oquendo, M.D., and J. John Mann, M.D.

It was created at **Columbia University**, in collaboration with the **University of Pennsylvania** and the **University of Pittsburgh**, with support from the **National Institute of Mental Health (NIMH)**.

1.2 Origin Context

The scale was originally developed for a **2007 NIMH-funded study (the Treatment of Adolescent Suicide Attempters [TASA] study)** examining treatments to decrease suicide risk among adolescents with depression. It emerged from more than 20 years of scientific study aimed at solving a critical problem in suicide research: the lack of a uniform, well-defined, and reliable way to classify suicidal ideation and behavior.

1.3 Foundational Instrument

The definitions of behavioral suicidal events used in the C-SSRS are based on **The Columbia Suicide History Form (CSHF)**, developed by **John Mann, MD** and **Maria Oquendo, MD** at the **Conte Center for the Neuroscience of Mental Disorders (CCNMD)**, New York State Psychiatric Institute. The CSHF itself derived from the **Columbia Classification Algorithm for Suicide Assessment (C-CASA)**, a retrospective method originally developed to classify adverse events reported in pediatric antidepressant clinical trials.

1.4 Historical Timeline

Year	Development
2003	Oquendo, Halberstam & Mann publish risk factor classification work underlying the Columbia Suicide History Form
2007	C-SSRS developed for the NIMH-funded TASA study on adolescent suicide attempters
2008	Copyright established — The Research Foundation for Mental Hygiene, Inc.
2011	Centers for Disease Control and Prevention (CDC) adopts the Columbia Protocol's definitions for suicidal behavior and recommends its use for data collection
2011	Posner et al. publish the landmark validation paper in the <i>American Journal of Psychiatry</i> : "The Columbia–Suicide Severity Rating Scale: Initial Validity and Internal Consistency Findings From Three Multisite Studies With Adolescents and Adults"
2012	The U.S. Food and Drug Administration (FDA) declares the Columbia Protocol the standard for measuring suicidal ideation and behavior in clinical trials
Present	Used across 45+ nations on six continents; translated into 150+ country-specific languages; used in hospitals, schools, faith communities, military, and correctional settings worldwide

1.5 Copyright & Access

© 2008 The Research Foundation for Mental Hygiene, Inc. The scale and its associated training are provided **free of charge** for use in community and healthcare settings, as well as in federally funded or nonprofit research. For reprints/official copies, contact: **Kelly Posner, Ph.D., New York State Psychiatric Institute, 1051 Riverside Drive, New York, NY 10032** (posnerk@nyspi.columbia.edu). The Columbia Lighthouse Project (cssrs.columbia.edu) is the official public-facing repository for all versions.

1.6 Key Reference

Posner, K., Brown, G. K., Stanley, B., Brent, D. A., Yershova, K. V., Oquendo, M. A., Currier, G. W., Melvin, G. A., Greenhill, L., Shen, S., & Mann, J. J. (2011). The

Columbia–Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. *American Journal of Psychiatry*, 168(12), 1266–1277.

2. Nature of the Scale

2.1 Classification

Parameter	Description
Type	Semi-structured clinical interview / suicide risk screening and severity-rating instrument
Category	Suicide-specific behavioral rating scale (not a general psychopathology or personality measure)
Approach	Categorical presence/absence (Yes/No) at each ideation and behavior level, PLUS ordinal severity, intensity, and lethality ratings
Format	Clinician-administered interview (primary); self-report version also available
Administration Time	Approximately 5 minutes for the core screening; longer for the Full/Lifetime-Recent version with full behavioral history
Training Required	The full/research version is intended for use by individuals with training in its administration; the Screener/Triage versions can be administered by anyone with no mental health training required (physicians, nurses, teachers, coaches, clergy, peer counselors, family members)
Age Range	Suitable for all ages; separate pediatric/children's wording is provided (as in the attached document), as well as adult wording

2.2 Guiding Philosophy

The C-SSRS is built on the principle that suicidal ideation and behavior exist on a **continuum of severity** — from passive wishes to be dead, through active ideation with increasing specificity of method/intent/plan, to actual suicidal acts of varying lethality. Rather than treating “suicidality” as a single yes/no construct, the scale **distinguishes between ideation and behavior** and further

grades each along a severity gradient, allowing clinicians to identify not just *whether* someone is at risk, but *how severe* and *how imminent* that risk is.

2.3 Two Core Domains Assessed

1. **Suicidal Ideation** — thoughts about suicide, rated across 5 levels of increasing severity (from passive wish to be dead to active ideation with specific plan and intent)
2. **Suicidal Behavior** — actions taken toward suicide, categorized into distinct behavioral subtypes (actual attempt, interrupted attempt, aborted attempt, preparatory acts, non-suicidal self-injury)

2.4 Important Clinical Note (from the Disclaimer, verbatim from the manual)

“This scale is intended to be used by individuals who have received training in its administration. The questions contained in the Columbia-Suicide Severity Rating Scale are suggested probes. Ultimately, the determination of the presence of suicidal ideation or behavior depends on the judgment of the individual administering the scale.”

This is a critical instruction: **the printed questions are prompts, not a rigid script**. The clinician must use judgment to determine whether a “yes” response truly reflects the construct being assessed, and may reword questions as needed for the respondent's developmental level, language, or clinical presentation.

3. Purpose & Applications

3.1 Primary Purpose

To systematically and reliably **identify, quantify, and track** the severity of suicidal ideation and behavior in an individual, in order to:

1. Determine whether someone is at risk for suicide
2. Establish the **severity** and **immediacy** of that risk
3. Gauge the **level of support/intervention** the person needs
4. Provide a common, standardized language for suicide risk across settings and providers

3.2 What the Scale Specifically Screens For

- Whether and when the person has thought about suicide (**ideation**, and how recently)
- What actions the person has taken — and when — to **prepare** for suicide
- Whether and when the person **attempted** suicide, or began an attempt that was **interrupted** by another person or **aborted** of their own volition

3.3 Clinical & Institutional Applications

Setting	Application
Psychiatric/Psychological Clinical Practice	Intake screening, ongoing risk monitoring, treatment planning, discharge planning
Emergency Departments	Rapid triage of patients presenting with self-harm or psychiatric crisis
Primary Care	Routine screening as part of depression/mental health assessment

Setting	Application
Clinical Trials (FDA Standard)	Mandated by the FDA since 2012 as the standard measure of treatment-emergent suicidal ideation/behavior in drug trials
Schools	Identifying at-risk students; used by teachers, counselors, and school psychologists
Military	Suicide risk screening among service members and veterans
Correctional Settings	Screening for suicide risk in prisons and probation settings
Faith Communities & Community Organizations	Lay-administered screening as part of gatekeeper training
Hospitals (Non-Psychiatric)	Screening medical/surgical inpatients for co-occurring suicide risk

3.4 Regulatory & Policy Endorsements

- **2011:** Adopted by the CDC for suicidal behavior classification and data collection
 - **2012:** Declared by the **FDA** as the standard for measuring suicidal ideation and behavior in clinical trials
 - Endorsed/recommended/adopted by numerous national and international health bodies, and used in **45+ nations** in **150+ languages**
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4. Versions of the C-SSRS

There are **four primary versions/components** of the C-SSRS, plus separate pediatric and adult wording tracks for each:

Version	Purpose	Length/ Time
 Screener/Triage Version	Ultra-brief screening (as few as 2, up to 6 questions) to quickly identify risk and determine need for further evaluation; can be used by anyone regardless of mental health training	~1-2 minutes
Lifetime/Recent (Baseline) Version	Comprehensive baseline assessment capturing an individual's entire lifetime history of suicidal ideation and behavior, plus a focus on the most recent episode	~5-10 minutes
Since Last Visit Version	Follow-up version used at each subsequent clinical contact to assess ideation/behavior that has occurred since the previous assessment (this is the version contained in the attached document — the Children's Since Last Visit form, version 6/23/10)	~5 minutes
Risk Assessment / Triage Page	A supplementary page used alongside any version to document protective factors, risk factors, clinical status, and the resulting risk level/care plan	Varies

4.1 Age-Specific Wording Tracks

Track	Population	Key Feature
Adult version	Adults	Standard adult-oriented probe wording
Pediatric/ Children's version	Children and adolescents	Simplified, developmentally appropriate wording (e.g., “make yourself not alive anymore” instead of clinical terms); this is the wording used in the attached document

4.2 Administration Format Variants

Format	Description
Clinician-administered (structured interview)	The primary and most extensively validated format — requires trained administration
Self-Report version	A self-administered paper/electronic version for settings where direct interview is not feasible
Clinical Practice Screener	A simplified version specifically designed for routine clinical practice (not research)

4.3 The Document in This Repository

The PDF provided (“**Columbia Suicide Rating Scale.pdf**”) is specifically the:

C-SSRS — Children's Since Last Visit (Version 6/23/10)

This means it is designed to be administered at a **follow-up visit** (not the first/baseline assessment) and captures suicidal ideation and behavior that has occurred **since the subject's last clinical contact**, using **child-appropriate language**. It is a 2-page form consisting of a **Suicidal Ideation** section, an **Intensity of Ideation** section, and a **Suicidal Behavior** section.

5. Structure & Content

(Based Directly on the Attached Document: C-SSRS — Children's Since Last Visit, Version 6/23/10)

The form contained in the repository consists of **2 pages** organized into **3 major sections**:

5.1 PAGE 1 — SUICIDAL IDEATION + INTENSITY OF IDEATION

A. Suicidal Ideation Section (5 Items, Skip-Structured)

#	Item Name	Core Construct
1	Wish to be Dead	Thoughts about a wish to be dead, not alive anymore, or wishing to fall asleep and not wake up
2	Non-Specific Active Suicidal Thoughts	General, non-specific thoughts of wanting to end one's life, WITHOUT thoughts of method, intent, or plan
3	Active Suicidal Ideation with Any Methods (Not Plan), without Intent to Act	Thoughts of suicide PLUS at least one method considered, but no specific plan and no intent to act
4	Active Suicidal Ideation with Some Intent to Act, without Specific Plan	Active suicidal thoughts + some reported intent to act on them (not merely "I have the thoughts but definitely will not act")
5	Active Suicidal Ideation with Specific Plan and Intent	Thoughts of suicide with a plan fully or partially worked out, AND some intent to carry it out

Skip logic built into the form:

- Ask Items 1 and 2 first
- If **both are negative (No)** skip directly to the "Suicidal Behavior" section

- If the answer to **Item 2 is “Yes”** proceed to ask Items 3, 4, and 5
- If the answer to **Item 1 and/or 2 is “Yes”** the “Intensity of Ideation” section must be completed

Each item is rated dichotomously: **Yes / No**, for the period **“Since Last Visit.”** If “Yes,” the administrator records a free-text description of the ideation.

B. Intensity of Ideation Section

This section is completed **only if Item 1 and/or 2 above was endorsed “Yes.”** It is rated with respect to the **single most severe type of ideation** endorsed (i.e., whichever of items 1–5 received the highest number that was endorsed “Yes”), where 1 = least severe and 5 = most severe.

Fields to complete:

- **Type # (1–5):** which ideation item was the most severe endorsed
- **Description of Ideation:** free-text description
- **Frequency:** “How many times have you had these thoughts?” — rated:
 - (1) Only one time
 - (2) A few times
 - (3) A lot
 - (4) All the time
 - (0) Don't know / Not applicable

(Note: The full/standard version of the C-SSRS additionally rates Duration, Controllability, Deterrents, and Reasons for Ideation within this “Intensity of Ideation” section — see Section 9 below for the complete 5-dimension intensity sub-scale as used in the standard Lifetime/Recent and Since Last Visit forms more broadly.)

5.2 PAGE 2 — SUICIDAL BEHAVIOR

Instruction at the top of this section: **“Check all that apply, so long as these are separate events; must ask about all types.”**

Category	Core Construct
Actual Attempt	A potentially self-injurious act committed with at least some intent to die as a result of the act. Intent does not have to be 100%; if there is <i>any</i> intent/desire to die associated with the act, it is considered an actual suicide attempt. There does not have to be any actual injury or harm — only the <i>potential</i> for injury/harm (e.g., pulling the trigger of a gun in the mouth that turns out to be broken still counts as an attempt)
Non-Suicidal Self-Injurious Behavior	Self-injury performed with no intent to die — done for other reasons (e.g., to feel better, or to make something else happen)
Self-Injurious Behavior, Intent Unknown	Self-injury where the presence/absence of suicidal intent could not be determined
Interrupted Attempt	When another person or outside circumstance stops the individual from starting the self-injurious act (e.g., pills in hand but stopped before ingesting; gun pointed at self but taken away before the trigger is pulled; noose around neck but stopped before hanging began)
Aborted Attempt	When the person begins to take steps toward a suicide attempt but stops themselves before engaging in any self-destructive behavior (distinguished from Interrupted Attempt only by <i>who</i> stops the act — self vs. external)
Preparatory Acts or Behavior	Acts or preparations toward imminently making a suicide attempt, beyond mere verbalization/thought — e.g., buying pills, purchasing a gun, giving possessions away, writing a suicide note
Suicidal Behavior (Present/Absent)	Whether ANY suicidal behavior (of the above types) was present during the assessment period
Completed Suicide	Whether the individual died by suicide

Lethality Ratings (Answered **ONLY** for Actual Attempts)

Actual Lethality/Medical Damage (0–5 scale):

Score	Description
0	No physical damage or very minor physical damage (e.g., surface scratches)
1	Minor physical damage (e.g., lethargic speech; first-degree burns; mild bleeding; sprains)
2	Moderate physical damage; medical attention needed (e.g., conscious but sleepy, somewhat responsive; second-degree burns; bleeding of a major vessel)
3	Moderately severe physical damage; medical hospitalization and likely intensive care required (e.g., comatose with reflexes intact; third-degree burns less than 20% of body; extensive blood loss but can recover; major fractures)
4	Severe physical damage; medical hospitalization with intensive care required (e.g., comatose without reflexes; third-degree burns over 20% of body; extensive blood loss with unstable vital signs; major damage to a vital area)
5	Death

Potential Lethality (answered **only if Actual Lethality = 0**, i.e., no medical damage occurred but the act could still have been lethal):

Score	Description
0	Behavior not likely to result in injury
1	Behavior likely to result in injury but not likely to cause death
2	Behavior likely to result in death despite available medical care

Example given in the manual: “put gun in mouth and pulled the trigger but gun fails to fire so no medical damage” or “laying on train tracks with oncoming train but pulled away before run over” — both would receive Actual Lethality = 0, but Potential Lethality would be rated 2 (behavior likely to result in death despite available medical care).

Additional fields recorded for behavior: **Total # of Attempts**, **Total # of Interrupted Attempts**, **Total # of Aborted Attempts**, and the **Date of the Most Lethal Attempt**.

6. Administration Procedure

6.1 Who Can Administer

Version	Administrator Requirement
Screener/Triage Version	No mental health training required — can be used by physicians, nurses, teachers, coaches, clergy, peer counselors, family members, or anyone in a position to identify at-risk individuals
Full/Lifetime-Recent and Since Last Visit Versions (research/clinical)	Individuals who have received specific training in administration of the scale, per the manual's disclaimer

6.2 Pre-Administration Considerations

1. **Establish rapport and privacy** — suicide-related questions should be asked in a private, non-judgmental setting
2. **Determine the assessment period** — for the Since Last Visit version, clarify with the respondent the timeframe (“since your last visit on [date]”)
3. **Use developmentally appropriate language** — the Children's version deliberately avoids clinical terminology (e.g., uses “make yourself not alive anymore” rather than “commit suicide”) to ensure comprehension
4. **Remember: the printed questions are suggested probes, not a rigid script** — the interviewer should feel free to rephrase for clarity while preserving the construct being assessed
5. **Determination of presence of ideation/behavior is a matter of clinical judgment** — a “yes” checkbox is not simply mechanical; the interviewer must judge whether the response genuinely reflects the item's definition

6.3 Step-by-Step Administration Sequence (Since Last Visit — Children's Version)

Step 1 — Ask Item 1 (Wish to be Dead) and Item 2 (Non-Specific Active Suicidal Thoughts).

Suggested probes: “Have you thought about being dead or what it would be like to be dead?” “Have you wished you were dead or wished you could go to sleep and never wake up?” “Have you had any thoughts about killing yourself?”

Step 2 — Apply skip logic:

- If **both Item 1 and Item 2 = No** proceed directly to the Suicidal Behavior section (Page 2)
- If **Item 2 = Yes** continue to ask Items 3, 4, and 5 in sequence
- If **Item 1 and/or Item 2 = Yes** the Intensity of Ideation section must be completed after finishing the ideation items

Step 3 — Ask Items 3, 4, 5 in order of increasing severity, using the specific probes provided:

- Item 3: *“Have you thought about how you would do that or how you would make yourself not alive anymore? What did you think about?”*
- Item 4: *“When you thought about making yourself not alive anymore, did you think that this was something you might actually do? This is different from having the thoughts but knowing you wouldn't do anything about it.”*
- Item 5: *“Have you decided how or when you would make yourself not alive anymore? Have you planned out (worked out the details of) how you would do it? What was your plan? When you made this plan, was any part of you thinking about actually doing it?”*

Step 4 — Complete the Intensity of Ideation section (if triggered):

- Identify the **single most severe ideation type** endorsed (highest numbered item marked “Yes”)
- Record the Type # (1–5) and a description

- Ask about frequency: “How many times have you had these thoughts?” and code accordingly (1=Only one time, 2=A few times, 3=A lot, 4=All the time, 0=Don't know/Not applicable)

Step 5 — Move to the Suicidal Behavior section (mandatory for ALL respondents, regardless of ideation answers):

- Ask about **all behavior subtypes** — do not skip any, “must ask about all types”
- For each subtype endorsed, record whether it occurred “**Since Last Visit**” and obtain a free-text description
- If an **Actual Attempt** occurred, complete the Actual Lethality/Medical Damage rating (0–5)
- If Actual Lethality = 0, additionally complete the Potential Lethality rating (0–2)
- Record the total number of attempts, interrupted attempts, and aborted attempts, and the date of the most lethal attempt

6.4 Special Notes on Inferring Intent (Behavior Section)

The manual provides explicit guidance for **inferring intent even when a respondent denies it**:

“Even if an individual denies intent/wish to die, it may be inferred clinically from the behavior or circumstances. For example, a highly lethal act that is clearly not an accident so no other intent but suicide can be inferred (e.g., gunshot to head, jumping from window of a high floor/story). Also, if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred.”

This means the clinician's judgment about the **objective lethality and circumstances of the act** can override a respondent's verbal denial of intent when classifying an event as an Actual Attempt.

6.5 Timing/Setting Guidance

- **Screening version:** ~1–2 minutes; can be used in any setting, requires no privacy protocol beyond standard clinical courtesy
 - **Since Last Visit / Lifetime-Recent versions:** ~5–10 minutes; should be conducted in a private clinical setting as part of a broader clinical interview or follow-up visit
 - Re-administer the Since Last Visit version **at every subsequent clinical contact** to capture new ideation/behavior occurring between visits
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7. Scoring System — Overview

The C-SSRS does **not** produce a single summary “total score” in the way personality or behavior checklists do. Instead, it generates **several independent, clinically meaningful scores**:

SUICIDAL IDEATION

Presence/Absence of each of 5 ideation types (Yes/No)

Ideation Severity Score = highest-numbered item endorsed “Yes” (1-5)

Intensity of Ideation = Frequency (+ Duration, Controllability, Deterrents,

Reasons for Ideation in the full/standard version)

SUICIDAL BEHAVIOR

Presence/Absence of each behavior subtype (Yes/No)

Total counts (# attempts, # interrupted, # aborted)

Actual Lethality/Medical Damage (0-5) -- for actual attempts only

Potential Lethality (0-2) -- only if Actual Lethality = 0

RISK STRATIFICATION

Composite clinical judgment integrating Ideation Severity + Recency +

Behavior + Risk/Protective Factors --> Low / Moderate / High Risk

7.1 Golden Rule of Scoring

Severity is always determined by the HIGHEST (most severe) item endorsed — never by summing or averaging. A person who endorses Items 1, 2, and 4 is scored as a “4” (Active Suicidal Ideation with Some Intent to Act), not as “1+2+4=7” or an average.

This “worst response counts” logic applies both within the Ideation Severity sub-scale and across the Behavior sub-scale (the most severe behavior type that occurred is what drives clinical urgency, not the number of behavior types).

8. Ideation Severity Sub-Scale (1–5)

This is the single most important score generated by the ideation portion of the scale. It is coded as the **highest-numbered item marked “Yes.”**

Level	Label	Definition	Sample Scored Response
1	Wish to be Dead	Passive thoughts about wanting to be dead or not wake up, WITHOUT any active thoughts of self-harm	“I sometimes think it would be easier if I just didn't wake up.”
2	Non-Specific Active Suicidal Thoughts	General thoughts of wanting to end one's life, with NO consideration of method/plan	“I've thought about killing myself, but I never think about how.”
3	Active Ideation with Method (No Plan/ Intent)	Thought of at least one method, but no specific plan and explicitly no intent to act	“I thought about overdosing, but I never worked out when or how, and I'd never actually do it.”
4	Active Ideation with Some Intent to Act	Thoughts of suicide + acknowledges that they <i>might</i> actually act on the thoughts (not merely “I think about it but would never do it”)	“When I think about it, part of me wonders if I actually would do it.”
5	Active Ideation with Specific Plan and Intent	Plan (fully or partially worked out) + some intent to carry it out	“I've thought about taking my father's pills at night when everyone's asleep, and I think about actually doing it.”

8.1 Clinical Significance of Each Level

Levels 1–2 are generally considered lower acute risk but still clinically significant and require follow-up and monitoring.

Level 3 introduces methodological specificity — while intent is explicitly denied, the presence of a considered method is a meaningful escalation.

Levels 4–5 represent the **highest-priority findings on the ideation scale** — per the Columbia Lighthouse Project's own triage guidance, *“a recent (past month) 'yes' to question 4 or 5 on ideation severity”* is one of the two most worrisome findings on the entire instrument (the other being any recent behavior).

Level 5 (specific plan + intent) constitutes an **acute psychiatric emergency** requiring immediate safety intervention.

8.2 Scoring Notes

- If the respondent's most severe endorsed item is, for example, Item 3, but Items 4 and 5 were subsequently asked (per skip logic) and answered “No,” the **Ideation Severity score is 3** — not affected by the negative answers to higher items
 - Item 2 acts as the **gatekeeper item**: if Item 2 = “No” (even if Item 1 = “Yes”), Items 3–5 are NOT asked, and Ideation Severity is capped at 1
 - If **both Item 1 and Item 2 = “No,”** the Ideation Severity score is effectively “0” (no ideation), and the interview proceeds directly to the Behavior section
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9. Intensity of Ideation Sub-Scale

The Intensity of Ideation section is completed **only** when Item 1 and/or Item 2 of the Ideation section was endorsed “Yes.” It characterizes the **most severe ideation type** identified above along additional dimensions. The **Children’s Since Last Visit form in this repository captures Frequency only**; the fuller Lifetime/Recent and standard Since Last Visit forms (per the official Columbia Lighthouse Project versions) capture **five dimensions** in total, all rated with respect to the single most severe ideation type:

Dimension	Question (Representative Wording)	Rating Scale
1. Frequency	“How many times have you had these thoughts?”	(1) Only one time (2) A few times (3) A lot (4) All the time (0) Don't know/Not applicable
2. Duration	“When you have the thoughts, how long do they last?”	(1) Fleeting – few seconds or minutes (2) Less than 1 hour/some of the time (3) 1–4 hours/a lot of time (4) 4–8 hours/most of day (5) More than 8 hours/persistent or continuous (0) Don't know/Not applicable
3. Controllability	“Could/can you stop thinking about killing yourself or wanting to die if you want to?”	(1) Easily able to control thoughts (2) Can control thoughts with little difficulty (3) Can control thoughts with some difficulty (4) Can control thoughts with a lot of difficulty (5) Unable to control thoughts (0) Does not attempt to control thoughts/Not applicable
4. Deterrents	“Are there things — anyone or anything (e.g., family, religion, pain of death) — that stopped you from wanting to die or acting on thoughts of suicide?”	(1) Deterrents definitely stopped you from attempting suicide (2) Deterrents probably stopped you (3) Uncertain if deterrents stopped you (4) Deterrents most likely did not stop you (5)

Dimension	Question (Representative Wording)	Rating Scale
		Deterrents definitely did not stop you (0) Does not apply
5. Reasons for Ideation	“What sort of reasons did you have for thinking about wanting to die or killing yourself? Was it to end the pain or stop feelings, or to get attention/revenge/a reaction, or both?”	(1) Completely to get attention, revenge, or a reaction from others (2) Mostly to get attention, revenge, or a reaction (3) Equally to get attention/revenge/reaction AND to end/stop the pain (4) Mostly to end/stop the pain (unable to cope) (5) Completely to end/stop the pain (unable to cope) (0) Does not apply

9.1 Clinical Interpretation of the Intensity Dimensions

- **High Frequency + Long Duration + Poor Controllability** = pervasive, hard-to-manage ideation higher acute risk and lower capacity for self-management between clinical contacts
- **Weak/absent Deterrents** (rated 4–5) = the person cannot identify factors holding them back from acting significant risk marker
- **Reasons skewed toward “ending pain” (rated 4–5)** rather than “attention/reaction” = ideation driven by genuine intent to escape suffering rather than interpersonal communication generally considered a higher-risk motivational profile

(Note: The attached Children's Since Last Visit form condenses this to Frequency alone, likely for brevity in routine follow-up visits; a comprehensive baseline/Lifetime-Recent assessment should capture all five dimensions.)

10. Suicidal Behavior Sub-Scale — Scoring

10.1 Behavior Subtype Hierarchy (Most to Least Severe)

For clinical/research classification purposes, behavior subtypes are generally considered in this order of severity:

1. **Completed Suicide** (fatal)
2. **Actual Attempt** (with Actual Lethality rated 0–5)
3. **Interrupted Attempt**
4. **Aborted Attempt**
5. **Preparatory Acts or Behavior**
6. **Non-Suicidal Self-Injurious Behavior / Self-Injurious Behavior, Intent Unknown** (scored separately — these are NOT counted as suicidal behavior for risk classification, but are documented because they carry their own clinical significance and may co-occur with or precede genuine suicidal behavior)

10.2 Step-by-Step Scoring of the Behavior Section

Step 1: For each subtype, record Yes/No for whether it occurred “**Since Last Visit.**”

Step 2: For every “Yes” on Actual Attempt, obtain:

- A free-text description of what was done
- The **Actual Lethality/Medical Damage** rating (0–5, see table in Section 5.2)

Step 3: If Actual Lethality = 0 (no physical damage occurred), ALSO obtain the **Potential Lethality** rating (0–2) to capture the fact that a low-damage outcome does not necessarily mean low-risk behavior (e.g., a gun that misfired).

Step 4: Tally:

- **Total # of Attempts** (all actual attempts during the period)

- **Total # of Interrupted Attempts**
- **Total # of Aborted Attempts**

Step 5: Identify and date the **Most Lethal Attempt** (the single actual attempt with the highest Actual Lethality score during the assessment period) — this is the attempt whose lethality code is entered in the “Most Lethal Attempt” field.

Step 6: Record whether **Suicidal Behavior** (of any type) was present during the period, and whether **Completed Suicide** occurred (obviously answered by a third party/chart review if so).

10.3 Distinguishing Interrupted vs. Aborted Attempts (Common Scoring Confusion)

	Interrupted Attempt	Aborted Attempt
Who stops the act?	An external person or circumstance	The individual stops themselves
Example	Person has pills in hand; a family member intervenes before ingestion	Person has pills in hand; decides on their own not to take them and puts them away
Clinical implication	Risk may remain high because the person did not choose to stop	May reflect some retained capacity for self-regulation, ambivalence, or a change of heart, though risk remains clinically significant

10.4 Distinguishing Suicidal from Non-Suicidal Self-Injury

The critical differentiator is **intent**:

- If there was **any** intent/wish to die associated with the act score as **Actual Attempt** (even minimal/partial intent counts)
- If there was **explicitly no intent to die**, and the behavior was performed for other reasons (e.g., emotional regulation, to feel something, to communicate distress) score as **Non-Suicidal Self-Injurious Behavior**

- If intent cannot be determined score as **Self-Injurious Behavior, Intent Unknown**, and flag for further clinical clarification

Per the manual's guidance on **inferring intent**, the clinician should not rely solely on verbal denial — objective lethality and circumstances can be used to infer suicidal intent even when the person denies it (see Section 6.4 above).

11. Risk Stratification & Triage

The C-SSRS itself does not output an automatic “risk score” — risk level is a **clinical determination** made by integrating the ideation severity, recency, behavior, and clinical/contextual factors. However, the Columbia Lighthouse Project provides an official triage convention, widely adopted across healthcare settings, which stratifies risk into **Low / Moderate / High**, as follows:

11.1 Official Columbia Protocol Triage Logic

Per the Columbia Lighthouse Project: “The most worrisome answers are the same in all settings and are a recent (past month) ‘yes’ to question 4 or 5 on ideation severity and/or any recent (past 3 months) behavior.”

11.2 General Risk Stratification Table

(as widely implemented in Columbia Protocol triage documents across hospital, school, and primary-care settings)

Risk Level	Ideation Criteria	Behavior Criteria
HIGH RISK	Ideation Severity Level 4 or 5 (some intent to act, or specific plan and intent) within the past month	ANY suicidal behavior (Actual, Interrupted, or Aborted Attempt, or Preparatory Acts) within the past 3 months
MODERATE RISK	Ideation Severity Level 2 or 3 (non-specific active thoughts, or thoughts of method without intent) within the past month ; OR Level 4/5 ideation more than a month ago	Suicidal behavior more than 3 months ago (lifetime history present but not recent)
LOW RISK	Ideation Severity Level 1 (passive wish to be dead) or no ideation	No lifetime history of suicidal behavior

HIGH RISK: Ideation Severity Level 4 or 5 within the past month, and/or ANY suicidal behavior (Actual, Interrupted, or Aborted Attempt, or Preparatory Acts) within the past 3 months. Warrants immediate safety intervention.

MODERATE RISK: Ideation Severity Level 2 or 3 within the past month (or Level 4/5 more than a month ago), and/or suicidal behavior more than 3 months ago. Warrants close monitoring and safety planning.

LOW RISK: Ideation Severity Level 1 or no ideation, and no lifetime history of suicidal behavior. Still warrants follow-up and exploration of underlying distress.

11.3 Important Clinical Caveats on Risk Stratification

1. **These categories are a starting framework, not a substitute for clinical judgment.** A person with a lifetime history of a highly lethal attempt, even if not recent, may warrant a higher risk classification than the table alone suggests.
2. **Risk level should always be considered alongside protective and risk factors** not directly captured by the C-SSRS items themselves — e.g., access to lethal means, social support, comorbid psychiatric illness, substance use, recent losses/stressors, and prior treatment engagement.
3. **Recency matters enormously.** The same ideation severity level carries substantially more urgency if endorsed “within the past week” versus “several months ago” — this is why the Since Last Visit and Lifetime/Recent forms both separately track *when* the most severe ideation/behavior occurred.
4. **Any behavior, regardless of lethality, elevates risk category** — even a low-lethality or “non-medically-damaging” actual attempt (Actual Lethality = 0) represents crossing the threshold from thought to action and should never be dismissed as low-risk simply because no injury resulted (recall: a broken gun that failed to fire still counts as an Actual Attempt).

11.4 The Risk Assessment / Triage Page (Supplementary Component)

In addition to the ideation and behavior sections, the full Columbia Protocol includes an optional **Risk Assessment Page**, used to document:

- **Risk Factors:** psychiatric diagnoses, prior attempts, family history of suicide, recent losses/stressors, access to lethal means, substance use, impulsivity, hopelessness
 - **Protective Factors:** social support, help-seeking behavior, religious/cultural beliefs against suicide, future orientation, responsibility to family/others
 - **Clinical Status:** current treatment engagement, medication compliance, recent changes in clinical presentation
 - **Resulting Risk Level and Care Plan:** the clinician's overall determination and the resulting safety/treatment plan (e.g., outpatient monitoring, safety planning, increased session frequency, hospitalization)
-

12. Interpretation Guidelines

12.1 General Principles

1. **Never interpret the C-SSRS score in isolation** — always integrate with clinical interview, history, mental status examination, and collateral information (family/caregiver report, especially for the pediatric version)
2. **The “worst response” governs urgency** — a single endorsement of Level 5 ideation or an Actual Attempt outweighs multiple lower-severity endorsements
3. **Recency is as important as severity** — always document *when* the most severe ideation/behavior occurred, not just *whether* it occurred
4. **A “No” on ideation does not rule out behavior** — the Suicidal Behavior section must ALWAYS be asked in full, regardless of ideation answers, because a person may have engaged in a preparatory act or an aborted/interrupted attempt without currently endorsing active ideation
5. **Consider the respondent's candor and capacity to report** — particularly with children, developmental level, rapport, and fear of consequences (e.g., hospitalization) may affect disclosure; corroborate with caregiver/collateral report wherever possible
6. **Document verbatim descriptions**, not just Yes/No codes — the free-text descriptions requested at every item are essential for clinical formulation and for future comparison at subsequent Since Last Visit assessments

12.2 Step-by-Step Interpretation Sequence

Step 1: Determine Ideation Severity (1–5) — identify the highest-numbered ideation item endorsed “Yes.”

Step 2: Determine Recency of that ideation — was it present at the time of assessment, within the past week, past month, or longer ago?

Step 3: Review Intensity of Ideation — frequency (and duration, controllability, deterrents, reasons, if the fuller form was used) to characterize how pervasive and hard-to-manage the ideation is.

Step 4: Review the full Suicidal Behavior section — identify the most severe behavior subtype endorsed and its recency, and note the lethality rating for any actual attempt.

Step 5: Cross-reference against the Risk Stratification table (Section 11) to arrive at a provisional Low/Moderate/High risk classification.

Step 6: Integrate contextual risk/protective factors (from the Risk Assessment Page or clinical interview) to finalize the risk determination and inform the safety/care plan.

Step 7: Compare against the prior assessment (if using the Since Last Visit version) — has severity, frequency, or behavior escalated, remained stable, or improved since the last visit? This trend is itself a critical piece of clinical information.

12.3 Illustrative Interpretation Patterns

Pattern	Interpretation
Item 1 “Yes,” Item 2 “No”	Passive death wish only; Ideation Severity = 1; generally lower acute risk but still warrants follow-up and exploration of underlying distress (e.g., depression, hopelessness)
Items 1–3 “Yes,” Item 4 “No”	Ideation Severity = 3; method has been considered, but intent to act is explicitly denied; monitor closely, as this reflects meaningful escalation beyond passive ideation
Items 1–5 all “Yes”	Ideation Severity = 5 (specific plan + intent); represents an acute risk requiring immediate safety intervention , regardless of behavior history
No ideation endorsed, but an Aborted Attempt is endorsed in the Behavior section	Despite a “clean” ideation screen at the moment of interview, this individual has a history of suicidal behavior and must be classified with elevated risk — underscores why the Behavior section must always be completed in full

Pattern	Interpretation
Actual Attempt with Actual Lethality = 0	Zero medical damage does NOT mean zero risk; if Potential Lethality is rated 2 (e.g., gun that misfired), this is functionally equivalent in risk terms to a near-fatal attempt

13. Report Writing Format

13.1 Standard Report Structure

- I. IDENTIFYING INFORMATION
- II. REASON FOR ASSESSMENT / REFERRAL
- III. TEST ADMINISTERED & RATIONALE
- IV. BEHAVIOURAL OBSERVATIONS
- V. RESULTS
 - A. Suicidal Ideation Findings (Items 1-5, Ideation Severity Score)
 - B. Intensity of Ideation
 - C. Suicidal Behavior Findings (all subtypes, lethality ratings)
 - D. Risk Stratification (Low/Moderate/High)
- VI. CLINICAL INTERPRETATION
- VII. RISK AND PROTECTIVE FACTORS
- VIII. SAFETY PLAN / RECOMMENDATIONS
- IX. FOLLOW-UP PLAN

13.2 Section-by-Section Writing Guide

I. Identifying Information: Name, age, sex, date of assessment, informant (self/caregiver), relationship of informant to patient, referral source, clinician's name and qualifications, and — for the Since Last Visit version — the date of the previous assessment and the period being covered.

II. Reason for Assessment: State clearly whether this is a baseline/intake assessment, a routine follow-up (Since Last Visit), or a crisis/urgent assessment prompted by a specific concern (e.g., disclosure by the child, parental concern, school report).

III. Test Administered & Rationale: *“The Columbia-Suicide Severity Rating Scale — Children's Since Last Visit version (C-SSRS; Posner et al., 2008) was administered to systematically assess the presence, severity, and recency of suicidal ideation and behavior occurring since the patient's previous clinical contact on [date].”*

IV. Behavioural Observations: Note the child's affect, cooperation, apparent candor, any hesitation or distress when discussing ideation/behavior items, and the presence/absence of a caregiver during administration (and whether this may have influenced disclosure).

V. Results — Writing Guidelines:

- Report **each ideation item individually** (Yes/No) with the associated free-text description where positive
- State the **Ideation Severity Score** clearly (e.g., "Ideation Severity = 3")
- Report the **Intensity of Ideation** findings (frequency, and duration/controllability/deterrents/reasons if assessed)
- Report **each behavior subtype individually**, with dates/descriptions, and the **lethality rating** for any actual attempt
- State the resulting **Risk Stratification** (Low/Moderate/High) and the specific criteria that drove that classification

VI. Clinical Interpretation: Integrate the C-SSRS findings with the broader clinical picture — psychiatric history, current symptoms, recent stressors, and any change from the prior assessment (for Since Last Visit administrations).

VII. Risk and Protective Factors: Explicitly list identified risk factors (e.g., access to means, recent stressor, prior attempt) and protective factors (e.g., strong caregiver relationship, help-seeking behavior, future orientation).

VIII. Safety Plan / Recommendations: Concrete, actionable next steps proportionate to the risk level — e.g., means restriction counseling with caregivers, safety planning, increased monitoring frequency, referral for psychiatric evaluation, or emergency-level intervention (crisis line, emergency department, hospitalization) for High Risk classifications.

IX. Follow-Up Plan: Specify when the C-SSRS Since Last Visit version will next be re-administered and what would trigger an earlier reassessment.

13.3 Language Guidelines for Report Writing

Instead of...	Write...
"The patient is suicidal"	"The patient endorsed active suicidal ideation with [specificity], classified as Ideation Severity Level [X]"
"No risk"	"No suicidal ideation or behavior was endorsed on the C-SSRS at this assessment; risk is classified as Low based on current findings"
"High risk — will attempt"	"Findings are consistent with High Risk classification per Columbia Protocol criteria (recent Level [4/5] ideation and/or recent suicidal behavior), warranting immediate safety intervention"
"Patient is lying"	"There was some inconsistency between the patient's self-report and collateral/caregiver information, warranting further clarification"

14. Sample Report Template

COLUMBIA-SUICIDE SEVERITY RATING SCALE (C-SSRS) — CHILDREN'S SINCE LAST VISIT

CLINICAL ASSESSMENT REPORT

Name: [Child's Name] | **Age:** [Age] | **Sex:** [M/F]
Date of Assessment: [Date] | **Date of Previous Visit:** [Date]
Informant(s): [Self / Caregiver Name and Relationship]
Assessed by: [Clinician Name, Qualification]

I. Reason for Assessment

[Routine follow-up per treatment protocol / Prompted by specific concern: describe]

II. Test Administered

Columbia-Suicide Severity Rating Scale — Children's Since Last Visit Version (C-SSRS; Posner et al., 2008), assessing suicidal ideation and behavior occurring since the patient's last clinical contact on [date].

III. Behavioural Observations

[Child's cooperation, affect, candor, any distress noted during administration; presence of caregiver]

IV. Results

A. Suicidal Ideation (Since Last Visit)

Item	Endorsed (Y/N)	Description (if Yes)
1. Wish to be Dead	[]	

Item	Endorsed (Y/ N)	Description (if Yes)
2. Non-Specific Active Suicidal Thoughts	[]	
3. Active Ideation with Method, No Intent	[]	
4. Active Ideation with Some Intent	[]	
5. Active Ideation with Specific Plan and Intent	[]	

Ideation Severity Score: [0-5]

B. Intensity of Ideation (completed if Item 1 and/or 2 = Yes)

Dimension	Finding
Most Severe Ideation Type	[1-5]
Frequency	[Only one time / A few times / A lot / All the time]

C. Suicidal Behavior (Since Last Visit)

Behavior Subtype	Endorsed (Y/ N)	Description	Date(s)
Actual Attempt	[]		
Non-Suicidal Self-Injurious Behavior	[]		
Self-Injurious Behavior, Intent Unknown	[]		
Interrupted Attempt	[]		
Aborted Attempt	[]		
Preparatory Acts or Behavior	[]		
Suicidal Behavior Present	[]		

Behavior Subtype	Endorsed (Y/N)	Description	Date(s)
Completed Suicide	[]		

If Actual Attempt endorsed:

- Actual Lethality/Medical Damage: [0-5]
- Potential Lethality (if Actual Lethality = 0): [0-2]
- Total # of Attempts: ____ | Total # Interrupted: ____ | Total # Aborted: ____
- Date of Most Lethal Attempt: ____

V. Risk Stratification

Based on the above findings, risk is classified as: [] **Low** [] **Moderate** [] **High**

Rationale: [State the specific ideation severity/recency and behavior/recency that drove this classification]

VI. Risk and Protective Factors

Risk Factors Identified	Protective Factors Identified
[e.g., access to means, recent stressor]	[e.g., caregiver support, help-seeking]

VII. Clinical Interpretation

[Narrative paragraph integrating the C-SSRS findings with clinical history, current presentation, and comparison to prior assessment if applicable]

VIII. Recommendations / Safety Plan

1. [e.g., Means restriction counseling with caregivers]
2. [e.g., Safety plan reviewed/updated with patient and caregiver]
3. [e.g., Referral for psychiatric evaluation]
4. [e.g., Increased session frequency]

5. [e.g., Emergency-level intervention if High Risk]

IX. Follow-Up

Next C-SSRS Since Last Visit administration scheduled for: [date/next visit]

Clinician's Signature: _____

Name & Qualification: [Name, Credentials]

Date: [Date]

15. Psychometric Properties

15.1 Landmark Validation Study

The primary validation of the C-SSRS was published by **Posner, K., Brown, G. K., Stanley, B., Brent, D. A., Yershova, K. V., Oquendo, M. A., Currier, G. W., Melvin, G. A., Greenhill, L., Shen, S., & Mann, J. J. (2011)** in the *American Journal of Psychiatry*, titled “*The Columbia–Suicide Severity Rating Scale: Initial Validity and Internal Consistency Findings From Three Multisite Studies With Adolescents and Adults.*”

15.2 Key Findings from the Validation Literature

- **Convergent and Divergent Validity:** The C-SSRS demonstrated good convergent validity with other multi-informant suicidal ideation and behavior scales, and good divergent validity (correctly distinguishing suicidal from non-suicidal constructs)
- **Sensitivity and Specificity:** High sensitivity and specificity for suicidal behavior classification when compared against another established behavior scale and an independent suicide evaluation board
- **Predictive Validity (Screeners Version):** Both the Screener's Ideation Severity and Behavior scales were associated with death by suicide within 1-week, 1-month, and 1-year follow-up periods in emergency department samples; the optimal cutoff on the ideation severity scale was associated with **more than 4 times the odds** of dying by suicide within 1 week (adjusted OR 4.7, 95% CI 1.5–14.8)
- **Foundational Reliability:** The underlying Columbia Classification Algorithm for Suicide Assessment (C-CASA), from which the C-SSRS behavioral definitions derive, demonstrated an overall inter-rater reliability of **.89**

15.3 Cross-Cultural and Population Validation

- Validated for use with **Spanish-speaking adolescents** in Latin American populations
- Validated for use in **New Zealand prisons and probation settings**
- Studied extensively across emergency department, military, primary care, school, and general hospital populations
- Available and linguistically/psychometrically validated in **150+ country-specific languages**

15.4 Regulatory Recognition as a Validity Marker

- **2011 CDC adoption** of the scale's definitions for national suicidal behavior surveillance and data collection is itself a strong indirect indicator of the instrument's perceived reliability and construct validity by a major public health authority
- **2012 FDA designation** as the standard instrument for measuring suicidal ideation/behavior in clinical trials represents the highest level of regulatory endorsement available for an instrument of this kind, based on the cumulative body of supporting psychometric evidence submitted and reviewed

15.5 Important Caveat Noted in the Literature

At least one independent evaluation (Military Suicide Research Consortium white paper, Gutierrez, 2011) has noted that, at the time of certain early evaluations, aspects of the scale's psychometric validation were still being established relative to its rapid adoption as a “first-line” clinical tool — underscoring the importance of using the C-SSRS as **one part of a comprehensive clinical risk assessment**, not a stand-alone diagnostic instrument (see Limitations, Section 16.2).

16. Advantages & Limitations

16.1 Advantages

#	Advantage
1	Free of charge for use in community, healthcare, and nonprofit/federally-funded research settings
2	Simple, plain-language questions — the Screener version can be administered by anyone, with no mental health training required
3	Captures the full spectrum of suicidal ideation and behavior — not just prior attempts, but ideation, preparatory acts, interrupted and aborted attempts, which many older instruments missed entirely
4	Distinguishes ideation from behavior and grades each along a clear severity gradient, rather than treating “suicidality” as a single construct
5	Strong regulatory endorsement — CDC-adopted definitions (2011) and FDA gold-standard status for clinical trials (2012)
6	Extensively cross-culturally validated — available in 150+ languages, used in 45+ nations
7	Efficient — core screening takes only minutes, reducing burden on both clinician and patient while still capturing clinically essential information
8	Scalable across settings — from brief lay-administered screening to full clinician-administered severity assessment, using the same underlying framework and definitions
9	Explicit behavioral definitions reduce ambiguity and improve inter-rater consistency in classifying complex or borderline events (e.g., interrupted vs. aborted attempts)
10	Built-in mechanism for tracking change over time via the Since Last Visit version, supporting longitudinal risk monitoring
11	Age-appropriate wording tracks (adult and pediatric) improve comprehension and validity of responses across developmental levels

16.2 Limitations

#	Limitation	Mitigation
1	Does not generate a single automated risk score — risk stratification requires clinical judgment integrating multiple sub-scores and contextual factors	Use the Risk Stratification table (Section 11) as a structured starting framework, supplemented by full clinical assessment
2	Relies heavily on self-report and disclosure — particularly with children, fear of consequences (e.g., hospitalization, parental reaction) may suppress honest reporting	Corroborate with caregiver/collateral report; build rapport; consider that “No” responses do not rule out risk if clinical suspicion remains high
3	Full/research version requires specific training to administer reliably per the manual's own disclaimer	Ensure administrators complete appropriate C-SSRS training (freely available via the Columbia Lighthouse Project) before independent use
4	Not a standalone diagnostic tool — cannot replace comprehensive psychiatric evaluation	Always integrate with full clinical interview, history, and mental status examination
5	“Yes/No” format may not capture nuance or ambivalence in complex clinical presentations	Use the free-text description fields fully; supplement with open-ended clinical interviewing
6	Since Last Visit version depends on accurate recall of the period since the prior visit, which may be imprecise, especially with children or with longer inter-visit intervals	Anchor the recall period with concrete reference points (e.g., “since your visit before the school holidays”)
7	Potential Lethality item is only rated when Actual Lethality = 0 , which could be overlooked in fast-paced clinical settings, understating true risk in “no-damage” events	Build a standard checklist/workflow reminder to always assess Potential Lethality whenever Actual Lethality is coded 0
8	Early independent evaluations flagged the pace of psychometric	Stay current with the evolving supporting evidence base (Columbia Lighthouse Project publishes an updated psychometric evidence

#	Limitation	Mitigation
	validation relative to rapid clinical adoption	book) and use the scale as one component of a multi-method risk assessment

17. References

17.1 Primary References

1. Posner, K., Brent, D., Lucas, C., Gould, M., Stanley, B., Brown, G., Fisher, P., Zelazny, J., Burke, A., Oquendo, M., & Mann, J. (2008). *Columbia-Suicide Severity Rating Scale (C-SSRS) — Children's Since Last Visit*, Version 6/23/10. New York State Psychiatric Institute / The Research Foundation for Mental Hygiene, Inc.
2. Posner, K., Brown, G. K., Stanley, B., Brent, D. A., Yershova, K. V., Oquendo, M. A., Currier, G. W., Melvin, G. A., Greenhill, L., Shen, S., & Mann, J. J. (2011). The Columbia–Suicide Severity Rating Scale: Initial validity and internal consistency findings from three multisite studies with adolescents and adults. *American Journal of Psychiatry*, 168(12), 1266–1277.
3. Oquendo, M. A., Halberstam, B., & Mann, J. J. (2003). Risk factors for suicidal behavior: Utility and limitations of research instruments. In M. B. First (Ed.), *Standardized Evaluation in Clinical Practice* (pp. 103–130). American Psychiatric Publishing.

17.2 Supplementary/Supporting References

1. The Columbia Lighthouse Project. (2023). *The Columbia Suicide Severity Rating Scale (C-SSRS): Supporting Evidence*. Columbia University Department of Psychiatry.
2. Gutierrez, P. M. (2011). *C-SSRS Evaluation White Paper*. Military Suicide Research Consortium.
3. Centers for Disease Control and Prevention. (2011). Definitions adopted from the Columbia Classification Algorithm for Suicide Assessment (C-CASA) for national suicide behavior surveillance and data collection.
4. U.S. Food and Drug Administration. (2012). Designation of the Columbia Protocol (C-SSRS) as the standard for assessing suicidal ideation and behavior in clinical trials.

17.3 Online Resources

1. The Columbia Lighthouse Project — Official Repository: <https://cssrs.columbia.edu>
2. Columbia University Department of Psychiatry — C-SSRS Research Lab: <https://columbiapsychiatry.org/research-labs/columbia-suicide-severity-rating-scale-c-ssrs>
3. Centers for Medicare & Medicaid Services. *Columbia Protocol Fact Sheet*: <https://www.cms.gov/files/document/coe-nf-columbia-protocol-final-508.pdf>

17.4 Contact for Official Reprints/Training

For official reprints of the C-SSRS or inquiries regarding training requirements, contact:

Kelly Posner, Ph.D.

New York State Psychiatric Institute

1051 Riverside Drive, New York, NY 10032

Email: posnerk@nyspi.columbia.edu

Appendix A: Quick Reference — Ideation Severity Levels

LEVEL 1 — Wish to be Dead (passive)

LEVEL 2 — Non-Specific Active Suicidal Thoughts (no method)

LEVEL 3 — Active Ideation + Method, NO intent to act

LEVEL 4 — Active Ideation + SOME intent to act (no specific plan)

LEVEL 5 — Active Ideation + SPECIFIC PLAN + intent [HIGHEST SEVERITY]

Appendix B: Quick Reference — Behavior Subtypes

Actual Attempt	-- self-injurious act + ANY intent to die
Non-Suicidal Self-Injury	-- self-injury, NO intent to die
Self-Injury, Intent Unknown	-- intent could not be determined
Interrupted Attempt	-- stopped by SOMEONE/SOMETHING ELSE
Aborted Attempt	-- stopped by SELF
Preparatory Acts	-- steps taken toward an attempt (means, note, giving things away)

Appendix C: Quick Reference — Lethality Scales

ACTUAL LETHALITY (0-5)

0 = No/minimal damage

1 = Minor damage

2 = Moderate damage

3 = Moderately severe damage

4 = Severe damage

5 = Death

POTENTIAL LETHALITY (0-2)

(rate ONLY if Actual Lethality = 0)

0 = Not likely to result in injury

1 = Likely to injure, not likely fatal

2 = Likely fatal despite medical care

Appendix D: Clinical Red Flags — Immediate Action Required

Regardless of overall risk classification, the following findings warrant **immediate** safety intervention:

Immediate Action Required When Present:

- Ideation Severity Level 4 or 5 **currently or within the past week**
- ANY Actual Attempt, regardless of lethality rating, **within the past 3 months**
- An Interrupted or Aborted Attempt **within the past month**
- Preparatory Acts or Behavior (e.g., a note written, means acquired) **currently in progress or very recent**
- A marked escalation in severity, frequency, or behavior since the last C-SSRS assessment (Since Last Visit version)

This guide was compiled using the attached official C-SSRS — Children's Since Last Visit (Version 6/23/10) document, cross-referenced with the Columbia Lighthouse Project's official publications, the Posner et al. (2011) validation study, and other peer-reviewed and regulatory sources. For actual clinical use, practitioners should complete official C-SSRS training and refer to the current official versions available at cssrs.columbia.edu.

Content adapted and rephrased for compliance with licensing restrictions on the original scale text.

— END OF GUIDE —

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